

Document 2

Manual Pain Points

Identified Operational Problems — Apollo Clinic

Talking Crooks IT Pvt Ltd

Internship Task 2 — Clinic Management Software

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1. Document Overview

Document ID	CMS-D2
Document Title	Manual Pain Points — Clinic Management Software
Source	Business Research (CMS-D1) — Apollo Clinic, Kanpur
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2. Pain Points Summary

The following pain points were identified by studying the current manual workflow at Apollo Clinic. Each pain point is categorised by domain, severity, and business impact.

#	Pain Point	Domain	Severity	Description	Business Impact
PP-01	Appointment Scheduling is Manual	Appointments	Critical	Patients call or walk in without prior booking. No system to check doctor availability. This leads to overbooking, long queues, and patient dissatisfaction.	Lost patients, poor reputation, revenue loss
PP-02	Patient Records Stored in Paper Files	Patient Management	Critical	All patient information — demographics, visit history, diagnoses — is recorded on paper. Files can be lost, damaged, or misplaced. No backup exists.	Data loss, legal risk, poor continuity of care
PP-03	Patient History Retrieval is Slow	Patient Management	High	Finding an old patient's records requires manual search through folders. During busy hours this can take 5–15 minutes, delaying consultations.	Wasted staff time, reduced patient throughput
PP-04	Billing is Not Centralised	Billing	Critical	Bills are calculated manually per visit with no link to patient history. No consolidated view of dues, payments, or outstanding amounts per patient.	Revenue leakage, billing errors, audit difficulty
PP-05	No Real-Time Reporting or Dashboard	Analytics	High	There is no way to view today's appointments, patient counts, or revenue at a glance. Monthly reports are compiled manually and are often inaccurate.	Uninformed management decisions, missed growth signals
PP-06	Prescription Errors Due to Handwriting	Prescription	High	Handwritten prescriptions are sometimes illegible, leading to pharmacists dispensing incorrect medicines. No digital copy is retained by the clinic.	Patient safety risk, liability exposure
PP-07	No Automated Patient Follow-Up	Communication	Medium	Follow-up reminders are sent manually via WhatsApp by the receptionist. Many patients miss follow-ups, reducing treatment effectiveness.	Poor patient outcomes, reduced revisit rate

#	Pain Point	Domain	Severity	Description	Business Impact
PP-08	Doctor Schedule Not Formally Tracked	Staff Management	Medium	Doctor availability, leaves, and substitutions are managed informally. Patients sometimes arrive to find their preferred doctor unavailable.	<i>Patient dissatisfaction, wasted appointments</i>
PP-09	Lab Reports Not Archived Digitally	Lab / Radiology	High	Printed lab reports are handed to patients with no digital retention. If a patient loses the report, retesting is the only option.	<i>Additional cost for patients, diagnostic delays</i>
PP-10	No Multi-User Role Access	System / Security	Medium	All clinic data is accessible by anyone present, with no role-based access control. Sensitive patient data is not protected from unauthorised viewing.	<i>Privacy violation, data security risk</i>

3. Quick Reference — Pain Points List

- PP-01: Appointment scheduling is manual — no online or telephonic booking system.
- PP-02: Patient records stored in paper files — risk of loss or damage.
- PP-03: Patient history retrieval is slow — 5 to 15 minutes per record search.
- PP-04: Billing is not centralised — manual calculations lead to errors and leakage.
- PP-05: No real-time reporting — management has no live visibility of operations.
- PP-06: Prescription errors due to illegible handwriting — patient safety risk.
- PP-07: No automated follow-up — patients miss appointments and treatment schedules.
- PP-08: Doctor schedule not formally tracked — leads to unavailability surprises.
- PP-09: Lab reports not digitally archived — if lost, retesting is required.
- PP-10: No role-based access control — patient data is not secured.